

1. Administrative & Study Identification

Full Study Title: A Randomized, Phase 2/3 Study Comparing BMS-986504 in Combination With Nab-paclitaxel and Gemcitabine Versus Placebo in Combination With Nab-paclitaxel and Gemcitabine in Participants With Untreated Metastatic Pancreatic Ductal Adenocarcinoma Harboring Homozygous MTAP Deletion **Study Title:** A Study Comparing BMS-986504 in Combination With Nab-paclitaxel and Gemcitabine Versus Placebo in Combination With Nab-paclitaxel and Gemcitabine in Participants With Untreated Metastatic Pancreatic Ductal Adenocarcinoma With Homozygous MTAP Deletion **Short Title/ Acronym:** MountainTAP-30 **Protocol Number:** CA240-0030 **NCT Number:** NCT07076121 **Posting ID:** 193831402896745096 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** BMS-986504 (a selective, MTA-cooperative PRMT5 inhibitor) + Nab-paclitaxel + Gemcitabine **Phase of Development:** Phase 2, Phase 3 **Trial Status:** Recruiting **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of BMS-986504 combined with nab-paclitaxel/gemcitabine compared to placebo plus nab-paclitaxel/gemcitabine by evaluating Progression-Free Survival (PFS) and Overall Survival (OS). **Secondary Objectives:** To evaluate Objective Response (OR), Disease Control Rate (DCR), Duration of Response (DOR), and Time to Objective Response (TTOR) by RECIST v1.1.

2.2 Background & Rationale MTAP-deletion (MTAP-del) is a genetic alteration found in 20–25% of patients with Pancreatic Ductal Adenocarcinoma (PDAC). Patients with homozygous MTAP-del cancers generally have a worse prognosis compared to those with MTAP-wild type cancers. BMS-986504 is a first-in-class MTA-cooperative PRMT5 inhibitor that selectively binds to and inhibits the PRMT5-MTA complex—a synthetic lethal target in MTAP-del cancer cells—while sparing MTAP-WT cells. This study aims to demonstrate that adding BMS-986504 to standard first-line chemotherapy improves survival outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (2:2:1:1 in Phase 2; 1:1 in Phase 3)

3.2 Planned Enrollment & Duration Target **\$N\$:** 470 participants
Number of Sites: 277 globally (across 24 countries) **Estimated Study Start:** 23-Oct-2025 **Estimated Primary Completion:** 03-May-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically or cytologically confirmed diagnosis of metastatic pancreatic ductal adenocarcinoma (PDAC). 3. Evidence of homozygous methylthioadenosine phosphorylase (MTAP) deletion or MTAP loss detected in tumor tissue. 4. Metastatic disease with at least 1 measurable lesion as per Response Evaluation Criteria in Solid Tumors version v1.1 (RECIST v1.1). 5. Participants must not have received any systemic anticancer treatments in the metastatic setting. If clinically indicated and as per investigator discretion, participants may receive up to 1 cycle of Nab-paclitaxel/Gemcitabine (nab-p/gem) in the metastatic setting and must have not progressed or required discontinuation due to intolerable toxicity. Initial cycle of nab-p/gem administered in the metastatic setting must have been completed prior to randomization.

4.2 Exclusion Criteria 1. Concurrent malignancy (present during screening) requiring treatment or a history of prior malignancy active within 2 years prior to screening. 2. Active brain metastases. 3. Select cardiac abnormalities. 4. Prior PRMT5 or MAT2A inhibitor treatment. (*Note: Other protocol-defined Inclusion/Exclusion criteria apply*).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants will receive the optimal Phase 2 dose of BMS-986504 (or matching placebo) along with standard dosing of Nab-paclitaxel and Gemcitabine (Gemcitabine ATC Code: L01BC05; Trade Names: Gemzar, Infugem). **Route of Administration:** Oral (BMS-986504) and IV infusions (Nab-paclitaxel and Gemcitabine). **Duration of Treatment:** Cycles will continue until disease progression, unacceptable toxicity, or consent withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for chemotherapy-induced toxicities (e.g., antiemetics, growth factors). **Prohibited:** Other systemic anticancer therapies, investigational agents, and concurrent use of strong CYP3A/CYP2C inducers/inhibitors (subject to protocol specifics).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Tumor Tissue MTAP-del confirmation, Baseline Imaging (CT/MRI), Labs
Baseline	Day 0	Randomization, First Dose of BMS-986504/Placebo + Chemo
Treatment Cycles	Per Protocol	Administration of Study Drug + Chemo, Safety Labs, PK/PD
Tumor Assessment	Every 8 Weeks	Efficacy Assessment (CT/MRI per RECIST 1.1)
End of Study	At Progression	Final Safety & Efficacy Assessment, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Definition: * **Progression-Free Survival (PFS) as assessed by RECIST v1.1:** Defined as the time between the randomization date and the date of progressive disease (PD) or death from any cause (whichever occurs first). * **Overall Survival (OS):** Defined as the time from the randomization date to the date of death from any cause.

Measurement Method: Radiographic imaging evaluated per RECIST v1.1 criteria (Phase 3 features dual primary endpoints evaluated by Blinded Independent Central Review [BICR]).

7.2 Secondary & Exploratory Endpoints * Objective Response (OR) as assessed by RECIST v1.1 * Duration of Response (DOR) as assessed by RECIST v1.1 * Time to Objective Response (TTOR) as assessed by RECIST v1.1 * Disease Control Rate (DCR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via continuous patient assessment, lab monitoring, and HCP physical exams at each cycle. **Serious Adverse Events (SAEs):** Must be reported to the sponsor within 24 hours of site awareness. **Data Monitoring Committee (DMC):** An independent DMC will evaluate safety and efficacy data at scheduled intervals, notably transitioning from Phase 2 to Phase 3.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all efficacy endpoints; Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target $N = 470$ is statistically powered to detect a clinically meaningful improvement in PFS and OS compared to standard-of-care chemotherapy. **Handling of Missing Data:** Censoring rules for PFS per FDA/EMA oncology guidelines; standard procedures for missing data (e.g., Kaplan-Meier methodology for time-to-event endpoints).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine on-site and centralized remote monitoring. **Audit Trail:** Blinded Independent Central Review (BICR) of radiographic images to eliminate investigator bias; rigorous eCRF data management.

11. Study Identifiers & Governance

Primary ID: CA240-0030 **Posting ID:** 193831402896745096 **Registry Number:** NCT07076121 **Trial Status:** Recruiting **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** MountainTAP-30 **Official Regulatory Title:** A Randomized, Phase 2/3 Study Comparing BMS-986504 in Combination With Nab-paclitaxel and Gemcitabine Versus Placebo in Combination With Nab-paclitaxel and Gemcitabine in Participants With Untreated Metastatic Pancreatic Ductal Adenocarcinoma Harboring Homozygous MTAP Deletion

12. Clinical Framework (PDAC Specific)

Disease Areas: Pancreatic Ductal Adenocarcinoma **Primary Condition:** Metastatic Pancreatic Ductal Adenocarcinoma (PDAC) **Preferred UMLS Name:** Adenocarcinoma of pancreas, metastatic **Diagnosis Threshold:** Confirmed homozygous MTAP deletion or MTAP loss in tumor tissue **Medical Methodology:** Targeted Synthetic Lethality (PRMT5 inhibition) combined with systemic chemotherapy **Optimization Target:** Progression-Free Survival (PFS) and Overall Survival (OS) prolongation

13. Study Procedures & Methodology

Management Pathway: First-line systemic anti-cancer therapy for metastatic PDAC **Education Protocol (HCP):** RECIST v1.1 imaging training, targeted PRMT5 inhibitor toxicity management **Education Protocol (Patient):** Informed consent detailing standard chemo side effects vs. investigational drug, symptom self-reporting **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for primary efficacy determination

14. Observation & Follow-Up Schedule

Total Duration: Until trial completion (Estimated 2029) **Onsite Visit Cadence:** Aligned with chemotherapy cycles (e.g., Weekly/Q3W depending on exact regimen) **Remote Monitoring Frequency:**

Continuous survival follow-up post-progression **Checklist Review**
Interval: Radiologic tumor assessment every ~8 weeks

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				